**جوستاف روسي انترناشيونال - مصر (EPE) ***

# طلب علاج أول مرة / متردد

**إسم المريض رباعياً :** مريم السيد على احمد
**المحافظة :** الجيزه **الحي / المدينة :** الهرم **العنوان :** 16 ش محمد عاطف
**رقم البطاقة :** 26609200103265 **رقم التليفون :** 01100665031 **تاريخ تسجيل الإستمارة :** 19-10-2026
**العمر(تاريخ الميلاد) :** 20-09-1966 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

**Name:** مريم السيد على احمد
**National ID:** 26609200103265

## تقرير طبي

**السيد**

**تحية طيبة وبعد,,**

| | |
|---|---|
| **إيماء للطلب المقدم من** | **بشأن تقرير طبي للمريض** |
| **بقسم:** | **رقم طبي:** |

**نتشرف بأن نوضح فيما يلي تقريرا عن حالته**

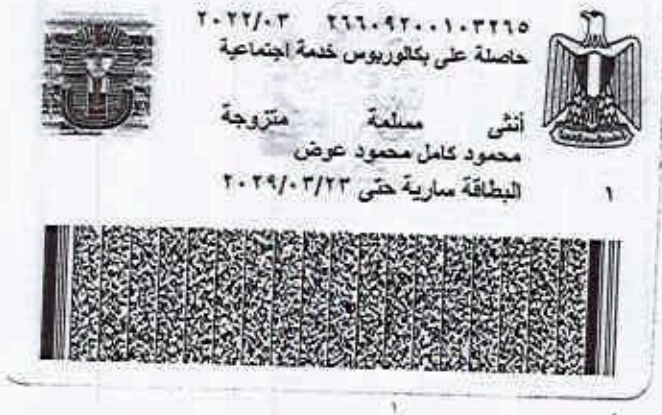
| | |
|---|---|
| **تاريخ وساعة الدخول :** -- --:-- yyyy-mm-dd | **تاريخ وساعة الخروج :** PM 01:17 05-07-2026 |

**وبعد الكشف والفحوصات والمتابعة وجد انة**

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

| | |
|---|---|
| **التاريخ :** **05-07-2026** | **الوقت :** **-- --:--** |
| | **الطبيب المعالج :** د/شيماء لاشين |




National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

**Patient's Name:** Mariam elsayed ali ahmed
**Date:** 2026-04-16
**Referred by:** Prof.Dr. cap+c
**Hospital:** **Goustave Roussey International Egypt**

## MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

**Cmpared to last study dated 5-1-2025 current study reveals;**

### I- CHEST:

- stationary course as regard the previously noted Left mild to moderate pleural effusion with underlying left lower lobar consolidation collapse.
- Right lower lobar thick atelectatic bands with related bronchectatic changes
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

### II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.
- Marked regression of the size and number of the previously noted few hepatic focal lesions, the largest still seen in the right lobe segment VIII measuring 8 mm (compared to 12 mm in the previous study), two of the lesions are hardly seen in today's study
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

### III- Bony skeleton:

Stionary coure as regard the Multiple sclerotic lesions with diffuse heterogenous density of left femoral head , bilateral iliac, sacral body, most lumbar and dorsal vertebral bodies, sternum, as well as ribs

**Opinion**

- **Regressive course regarding the size and number of the previously noted hepatic focal lesions.**
- **Stationary course regarding the rest of the study.**

***Dr.Asser Ahmed***

***Dr.Asser Ahmed***

CT Scan
[[DOCTYPE:CT_SCAN]]

| | |
|---|---|
| **Name:** مريم السيد على احمد<br>**MR. :** 2386 | |

Detailed Medical Report

| | |
|---|---|
| **السيد** | |

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | | | |
|---|---|---|---|
| **تاريخ وساعة الدخول :** | | **تاريخ وساعة الخروج :** | |

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**The patient is a 60-year-old individual, postmenopausal, with a history of left breast cancer treated with breast-conserving surgery in 2018 followed by adjuvant aromatase inhibitor therapy. The disease has progressed to metastatic breast cancer involving bone, pleura, and liver, with prior systemic regimens including Faslodex, Ibrance (8 cycles), docetaxel, radiotherapy to the hip, humeral fracture, and subsequent cycles of capecitabine and carboplatin-gemcitabine (Gemzar) initiated in October 2025. Comorbidities are not listed.**

**The most recent biopsy of the metastatic disease demonstrated estrogen receptor 8/8, progesterone receptor 6/8, and HER2-negative status. Imaging performed on 25 April 2026 showed a bone scan with residual osseous lesions in the shoulder, sternum, both clavicles, dorsal vertebrae, and left knee. A CT scan on 16 April 2026 reported a reduction in size of a previously noted high-frequency lesion, the largest measuring 8 mm compared with 12 mm earlier. Subsequent CT on 19 July 2026 identified a minimal left pleural effusion and several high-frequency lesions in both lung lobes, the largest 10 mm, along with stable osseous disease. The patient continues on gemcitabine-carboplatin, now completing 13 cycles, and has recently developed multiple new skin nodules over the presternal area, neck, and both breasts.**

**The referring team recommended baseline CT of the chest, abdomen, and pelvis with tumor markers, and a biopsy of the skin nodules for ER, PR, and HER2 assessment. The multidisciplinary team agreed to initiate aromatase inhibitor therapy with aromazone plus everolimus (Afinitor) and to reassess the patient after four months with repeat CT imaging and tumor marker evaluation.**

**تحريرا في ,**

| | | | |
|---|---|---|---|
| **التاريخ :** | **23-09-2026** | **الوقت :** | |

**الطبيب المعالج :**

**يعتمد ,**

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

صحة مصر

# MDT FORM 29/10/2024 G

| | | | |
|---|---|---|---|
| Name | مريم السيد | MRN | C.J / 2VJ |
| Age | 58 | ID | ٩٦٦٠٩٤٠١٠٢٤٩٠ |
| Menopausal status | Post | Phone | ٠١١٠٠٦٦٥٠٣١ |
| NRS SCORE | | Marital status | |
| BMI | | Family history | |
| CO morbidity | HTN | REASON OF MDT | |

## Clinical Data

BX:

| ER: | PR: | HER 2: | KI67: |
|---|---|---|---|
| 8/8 | 6/8 | -Ve | 35 % |

Sonomammography:

2019) - Known case of Rt breast Ca, pT2N1 Adjuvant At 35 Years

Mets Workup:

- 2024 → Bone Mets → Zoledex + Ibrance x 8 Months
- 11/2025 → O.P → start Tamoxifen + Zoledex + Palliative RT
- → Rt Humerus Fracture → CT → O.P. → Palliative RT to shoulder, humerus

R MBC to bone L

Ps II to Ps III

TNM

Recommendations:

Monthly → Zoledex + Navelbine → Zoledex then O.P

→ Not undergone to Fixation After orthopedic consultation

↳ P. Coordios to Performance ... surgery will Not Indicated

Non Contrast CT CAP 8/10/2024 → Newly developed Mild pleural effusion

" HFL

More sclerosis of bone lesions

MDT OUTCOME & PLAN

Recommended Revision → CTs & Radiology Cyclo 1 Cycle

3rd Cycle → Zoledex + Navelbine

Then Zoledex each x3 cycles

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|---|---|---|---|---|---|
| | | | | | |

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. PhD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مريم السيد على
Analysis ID : 385011
Age : 53 Y
Gender : Female
Referred By Prof. Dr. : محمد فلان
Clinical Diagnosis : Left breast mass
Nature of Specimen : Wide local excision & axillary clearance
Receiving Date : 10/06/2019
Patient ID : 307584

# PATHOLOGY REPORT

## Gross :

Two specimens were received designated as:
1- Wide local excision specimen measures 8x7x4.5 cm. Cut section revealed firm gritty tumour tissue with ill defined borders, measured 3.6x3.5x2.8 cm, 0.5 cm away from the nearest margin. A separate nodule measured 1x1 cm was received along.
2- An axillary fibrofatty tissue measuring 12x9x2 cm. Dissection revealed 18 lymph nodes, the largest 3x1.6 cm, with grayish cut section.

## Microscopic :

Sections examined from specimen (1) received revealed breast tissue showing infiltrating tumour tissue formed of irregular groups of malignant ductal cells showing moderate nuclear pleomorphism and hyperchromasia. There is moderate attempt at tubular formation. There is moderate number of mitotic figures. There are few tumour embol. No tumoural necrosis. There is marked desmoplasia. No intraductal component
The surrounding breast tissue showed acanosis.
Surgical margins are free.
Sections examined from the axillary nodes revealed metastatic deposits in 1 out of 18 nodes with no capsular infiltration.

## Diagnosis :

**Left breast mass, Wide local excision & axillary clearance, INFILTRATING DUCT CARCINOMA GRADE II, NO INTRADUCTAL COMPONENT, FREE SURGICAL MARGINS, AXILLARY LYMPH NODE METASTASIS, L.N. 1 / 18, WITH NO CAPSULAR INFILTRATION (N1).**
**TUMOR STAGE: pT2 pN1**

12/06/2019



عمارة رنسيس - بلوك أ - ميدان رمسيس - الدور الرابع و الخامس و السابع
تليفون : ٢٥٧٩٠٤٢٥ - ٢٥٧٤٢٧٤٤ - ٢٥٧٧٢٢٥٢ فاكس : ٢٥٧٧٢٢٥٦

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL CANCER CAMPUS

| | |
|---|---|
| **Name : mariam al sayed ali** | **Date : 05-01-2026 02:35** |
| **Branch : Portable** | **Medical No. : GRI-97301** |
| **Ref.Physician Prof :** | **Ins.Company :** |

## MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

### I- CHEST:

- Left moderate pleural effusion with underlying left lower lobar consolidation collapse.
- Right lower lobar consolidation with surrounding area of linear fibrotic bands
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

### II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. Multiple hypodense focal lesions seen in both lobes, the largest measures 12 mm, all showed marginal enhancement
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

### III- Bony skeleton:

Multiple sclerotic lesions with diffuse heterogenous density of bilateral iliac, sacral body, lumbar and dorsal vertebral bodies, sternum, as well as ribs

## Opinion:

- Known case of breast cancer.
- Moderate left pleural effusion with underlying left lower lobar consolidation collapse.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

| | |
|---|---|
| Name : mariam al sayed ali | Date : 05-01-2026 02:35 |
| Branch : Portable | Medical No. : GRI-97301 |
| Ref.Physician Prof : | Ins.Company : |

- Right lower lobar consolidation.
- Multiple hepatic and osseous metastatic lesions

*Dr. Asser Ahmed*

*Dr. Asser Ahmed*

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

| | |
|---|---|
| Name : Mariam Al Sayed Ali Ahmed | Date : 24-12-2025 02:47 |
| Branch : Portable | Medical No. : GRI-97301 |
| Ref.Physician Prof : | Ins.Company : |

## FOLLOW UP MR EXAMINATION OF THE BRAIN

**Technique:**

Axial T1, T2 ,FLAIR, diffusion & ADC WIs
Coronal T2WIs
Sagittal T1WIs

**Findings:**

- **Compared to the previous report done on 30/01/2025, the current study showed no interval changes**
- A stationary focus that noted in the left superior frontal periventricular cerebral white matter. It elicits high signal intensity in T1, T2 and FLAIR images. It is not surrounded by edema and exert no mass effect with no diffusion restriction.
- Mildly dilated cortical sulci, sylvian fissures and extra-axial CSF spaces as well as the cerebral ventricular system.
- No other areas of abnormal signal intensity.
- No evidences of mass lesions or abnormal enhancement seen.
- No intra-or extra-axial areas of blood intensity.
- Normal cerebellum, brain stem & cervico – medullary junction.
- Normal sellar region.
- No intracerebral or extra axial hematomas

**OPINION:**

**Stationary course regarding a small left frontal periventricular focus of non-specific altered signal intensity, likely ischemic changes.**
**Age matched involutional brain changes.**

Best regards
Dr. Jaber Gliwan
Dr Mahmoud Abdelmohsen.FEBR.FABR

M.R.I
[[DOCTYPE:MRI]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

| | |
|---|---|
| Name : Mariam Al Sayed Ali Ahmed | Date : 24-12-2025 02:47 |
| Branch : Portable | Medical No. : GRI-97301 |
| Ref.Physician Prof : | Ins.Company : |

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**Age matched involutional brain changes.**

Best regards
Dr. Jaber Gliwan
Dr Mahmoud Abdelmohsen.FEBR.FABR

M.R.I
[[DOCTYPE:MRI]]




National ID · Front
[[DOCTYPE:ID_FRONT]]




DMS
Cmc ←
جيمز، كاربو ←

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لتنمية الصحة مصر

08/26/2026

# MDT FORM

| | | | |
|---|---|---|---|
| **Name:** | Mariem El-Sayed Ali Ahmed | **MRN:** | 2386 |
| **Age:** | 60 Y | **ID:** | 26609200103265 |
| **Menopausal Sta** | postm | **Phone:*** | 01100665031 |
| **NRS SCORE:** | | **Marital Status:** | |
| **BMI:** | | **Family History:** | |
| **Gender** | | | |
| **CO Morbidity:** | | **Reason Of MDT:** | newly developped skin nodules |
| **Surgical History:*** | left bcs | **ECOG Performance Status** | ○ 0 ○ 1 ○ 2 ○ 3 ◉ 4 |

## Clinical Data:

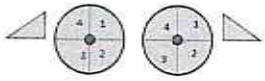
**BX:**

**ER:** **PR:** **HER2:** **KI67:**

**History:**

POSTM MBC TO BONE ,PLEURA ,LIVER ER8/8 PR6/8 HER 2-VE
HISTORY :
LEFT BREAST CANCER LEFT BCS 2018 ADJ AI
2024 BONE METS ST FASOLDEX IBRANCE X8 CYCLES DP TAXOL RT HIP AND RT HUMERAL FRACTURE DP XELODA NAV X4 CYCLES DP
CTS10/2025 NEWLY DEV HFL MILD PLEURAL EFF ST GEMZAR CARBOPLATIN
from 10/2025
bone scan on 25/4/2026 RD of residual met osseouslesions shoulder ,sternum both clavicles dl vert both si j left knee
ctson 16/4/2026 RD of no ,size of previous hfl seg 8 largest m 8 mm comp to 12 mm
cts 19/7/2026 left minimal pleural eff ,few hfl in both lobes largest 10mm sd of osseous lesions
pt continues gemzar carboplatin till now rec 13 cycles
recenmtly multiple newly developped skin nodules presternal area ,neck both breasts
for mdt

R L



T N M

| Right Side | | | | Left Side | | | |
|---|---|---|---|---|---|---|---|
| Q1 | Q2 | Q3 | Q4 | Q1 | Q2 | Q3 | Q4 |
| | | | | | | | |

**Clinical Staging:**

**Pathological Staging:**

**Sonomammography:**

**Biomarker/Molecular**

**Mets Workup:**

**Recommendations:**

**MDT Outcome & Plan**

BASE LINE CT CAP + TUMOR MARKER
BIOPSY FROM SKIN NODULE ER PR HER2
STARTE AI AROMAZINE + AFINITOR
EVALUATION AFTER 4 MONTHS BY CT S AND TM

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|---|---|---|---|
| د محمد كيلاني | د محمد كفراوي | | |

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لتنمية الصحة مصر

08/26/2026

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| **BMI:** | | **Family History:** | |
| **Gender** | | | |
| **CO Morbidity:** | | **Reason Of MDT:** | newly developped skin nodules |
| **Surgical History:*** | left bcs | **ECOG Performance Status** | ○ 0 ○ 1 ○ 2 ○ 3 ◉ 4 |

## Clinical Data:

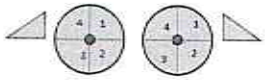
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for mdt

R L



T N M

| Right Side | | | | Left Side | | | |
|---|---|---|---|---|---|---|---|
| Q1 | Q2 | Q3 | Q4 | Q1 | Q2 | Q3 | Q4 |
| | | | | | | | |

**Clinical Staging:**

**Pathological Staging:**

**Sonomammography:**

**Biomarker/Molecular**

**Mets Workup:**

**Recommendations:**

**MDT Outcome & Plan**

BASE LINE CT CAP + TUMOR MARKER
BIOPSY FROM SKIN NODULE ER PR HER2
STARTE AI AROMAZINE + AFINITOR
EVALUATION AFTER 4 MONTHS BY CT S AND TM

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|---|---|---|---|
| د محمد كيلاني | د محمد كفراوي | | |

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لتنمية الصحة مصر

08/26/2026

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| **Gender** | | | |
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| **Surgical History:*** | left bcs | **ECOG Performance Status** | ○ 0 ○ 1 ○ 2 ○ 3 ◉ 4 |

## Clinical Data:

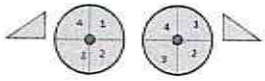
**BX:**

**ER:** **PR:** **HER2:** **KI67:**

**History:**

POSTM MBC TO BONE ,PLEURA ,LIVER ER8/8 PR6/8 HER 2-VE
HISTORY :
LEFT BREAST CANCER LEFT BCS 2018 ADJ AI
2024 BONE METS ST FASOLDEX IBRANCE X8 CYCLES DP TAXOL RT HIP AND RT HUMERAL FRACTURE DP XELODA NAV X4 CYCLES DP
CTS10/2025 NEWLY DEV HFL MILD PLEURAL EFF ST GEMZAR CARBOPLATIN
from 10/2025
bone scan on 25/4/2026 RD of residual met osseouslesions shoulder ,sternum both clavicles dl vert both si j left knee
ctson 16/4/2026 RD of no ,size of previous hfl seg 8 largest m 8 mm comp to 12 mm
cts 19/7/2026 left minimal pleural eff ,few hfl in both lobes largest 10mm sd of osseous lesions
pt continues gemzar carboplatin till now rec 13 cycles
recenmtly multiple newly developped skin nodules presternal area ,neck both breasts
for mdt

R L



T N M

| Right Side | | | | Left Side | | | |
|---|---|---|---|---|---|---|---|
| Q1 | Q2 | Q3 | Q4 | Q1 | Q2 | Q3 | Q4 |
| | | | | | | | |

**Clinical Staging:**

**Pathological Staging:**

**Sonomammography:**

**Biomarker/Molecular**

**Mets Workup:**

**Recommendations:**

**MDT Outcome & Plan**

BASE LINE CT CAP + TUMOR MARKER
BIOPSY FROM SKIN NODULE ER PR HER2
STARTE AI AROMAZINE + AFINITOR
EVALUATION AFTER 4 MONTHS BY CT S AND TM

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|---|---|---|---|
| د محمد كيلاني | د محمد كفراوي | | |

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لتنمية الصحة مصر

08/26/2026

# MDT FORM

| | | | |
|---|---|---|---|
| **Name:** | Mariem El-Sayed Ali Ahmed | **MRN:** | 2386 |
| **Age:** | 60 Y | **ID:** | 26609200103265 |
| **Menopausal Sta** | postm | **Phone:*** | 01100665031 |
| **NRS SCORE:** | | **Marital Status:** | |
| **BMI:** | | **Family History:** | |
| **Gender** | | | |
| **CO Morbidity:** | | **Reason Of MDT:** | newly developped skin nodules |
| **Surgical History:*** | left bcs | **ECOG Performance Status** | ○ 0 ○ 1 ○ 2 ○ 3 ◉ 4 |

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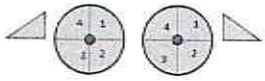
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BASE LINE CT CAP + TUMOR MARKER
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| د محمد كيلاني | د محمد كفراوي | | |

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لتنمية الصحة مصر

08/26/2026

# MDT FORM

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| **Name:** | Mariem El-Sayed Ali Ahmed | **MRN:** | 2386 |
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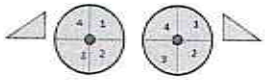
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|---|---|---|---|
| د محمد كيلاني | د محمد كفراوي | | |

[[DOCTYPE:BONE_SCAN]]
Bone Scan

د . هاني أحمد سامي
دكتوراه الأشعة التشخيصية
أستاذ بكلية طب القصر العيني

مركز الطاهرة للأشعة
منذ عام ١٩٩٦

TRC
TAHRA RADIOLOGY CENTER
EST. 1996

د . عصام علي الشيخ
دكتوراه الأشعة التشخيصية
أستاذ بكلية طب القصر العيني

**Name: Mrs. Maryam Elsayed Ali Ahmed**

**Date: 28/6/2025** ***ID:1120034728*** ***Acc.:5140178***

**Referring Prof. Dr: Hospital Harm! /** نقابة تجاريين القاهرة

# Tc 99m MDP whole body bone scan

## Technique:

- Whole body images were acquired 180 minutes following IV injection of 20 mCi of the tracer.

## Findings:

- Multiple focal and diffuse spots of active tracer uptake are seen in skull bones, mandible, both shoulders, right humerus, sternum, both scapulae, both clavicles, multiple bilateral ribs, multiple levels at most of dorsal and lumbar spine, sacrum, both sacroiliacs, both iliac bones, both acetabulae, both ischial bones, both femora, around left knee joint.

## Opinion:

- ***Widespread metastatic osseous deposits, for follow up bone scan.***

**Truly yours**
**Prof. Dr. Ahmed Kandeel. MD**
**Prof. of Nuclear Medicine, faculty of medicine, Cairo University**

Typed: M.sh 29/06/25 8:17:45 AM



مركز فيصل : ١١١ شارع الملك فيصل - الهرم - الجيزة
مركز حدائق الأهرام : ٢٠ م يمين البوابة الثالثة ( منفرع )
فرع المنصورة ( ميت غمر ) : ٧٣ شارع الحرية - أمام مصنع الغزل والنسيج
فرع المنيا : المنيا الجديدة - مول سيتي سكيب - الدور الأرضي

ACCREDITED MSCB-113
HOTLINE 19173
G-CERT SYSTEM SERVICE ISO 9001:2015 Client No: GIEG-0152-QC

المركز الرئيسي : ٢٥١ شارع بورسعيد - السيدة زينب - القاهرة
مركز مصر الجديدة : ٨٧ شارع العقريزي - الخليفة المأمون - روكسي
مركز المقطم : مجمع عيادات ش ٩ - أمام مسجد الفتح لذى المقطم - خلف بترول مويل
الفرع الإداري : أول فيصل ١٨١ برج السلام أمام كوبري فيصل

[[DOCTYPE:CT_SCAN]]
CT Scan



**Name: Mrs. Maryam El-Sayed Ali Ahmed**

**Date: 26/9/2026** ***ID:1120034728*** ***Acc.:5305788***

**Referring / نقابة تجاريين القاهرة**

## Non-Contrast MSCT scan examination of the Chest, Abdomen and Pelvis, revealed:

**Clinical Background: Known case of breast cancer**

**No previous imaging studies available, limited comparison made to the submitted report dated19/07/2026:**

## I) Chest:

- Applied right CVC with the distal tip projecting over the cavo-atrial junction.
- Diffuse prominent interlobular septal and fissural thickening of the left lung.
- The anterior left lung and lingula show peripheral subpleural reticulations, likely post radiotherapy changes.
- Associated left sided volume loss and mild elevation of the left hemi-diaphragm.
- RUL anterior segment 4mm nodule is noted, indeterminate.
- Right lower apical thick fibrotic bands with mild bronchiectatic and subsegmental atelectatic changes.
- Still noted left minimal to mild pleural effusion with underlying left basal thick pleuro-parenchymal thick scarring and subsegmental atelectatic changes.
- No right pleural collection.
- No newly developed consolidation, cavitations or masses.
- No pathologically enlarged mediastinal lymph nodes.
- No gross cardiomegaly. No pericardial effusion.
- Bilateral elastofibroma dorsi, more sizable on the right side.
- Left breast parenchyma shows few calcific foci.



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ACCREDITED MSCB-113
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الفرع الإداري : أول فيصل ١ ٤٨ برج السلام أمام كوبرى فيصل

[[DOCTYPE:CT_SCAN]]
CT Scan



**Name:** **Mrs. Maryam El-Sayed Ali Ahmed**

## II) Abdomen and Pelvis:

- Average size of the liver with homogenous parenchymal attenuation. Few inconspicuous focal lesions are identified.
- Normal non-contrast CT appearance of the spleen, pancreas and both kidneys.
- Atheromatous calcifications of the aorta.
- No gross abdominal or pelvic lymph node enlargement.
- IUD is seen in utero, no gross uterine or adnexal mass lesions.
- No abdominal ascites or pelvic collections.
- Normal appearance of the urinary bladder.
- Marked asymmetric atrophy of the right sided pelvic musculature.
- The scanned bony cage and vertebrae show multiple heterogenous osseous texture with metastatic deposits, predominantly sclerotic in appearance.

## Conclusion:

- **Diffuse prominent interlobular septal and fissural thickening involving the left lung, raising the possibility of lymphatic tumor involvement.**
- **Superimposed post radiotherapy fibrotic changes in the anterior lung/lingula with associated left sided volume loss.**
- **Left minimal pleural effusion.**
- **Few inconspicuous hepatic focal lesions, for contrast enhancment assessment.**
- **For correlation with previous unavailable prior imaging studies.**

**Truly yours;**
**Dr. Enas Samy, MD**

Printed by: wa 12/10/25 1:28:19 PM



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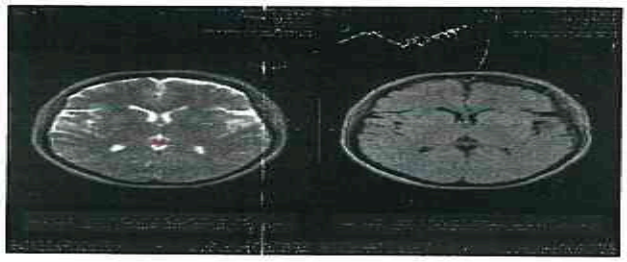
[[DOCTYPE:MRI]]
M.R.I

| | |
|---|---|
| **Patient's Name:** | **Maryam El-Sayed Ali Ahmed** |
| **Date:** | **2026-07-19** |
| **Referred by:** | Prof.Dr. **no doctor** |
| **Hospital:** | **Goustave Roussey International Egypt** |

## MRI EXAMINATION OF THE BRAIN

**MR Technique:**

- Axial T1, T2 and FLAIR WI's
- Sagittal T1WI
- Coronal T2 WI
- Axial,sagittal and coronal T1WIs with contrast.
- DWI & ADC sequence.



**MR findings:**

- Widened cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces with prominence of the ventricular systems.
- Two tiny non-enhancing foci of altered signals are seen in the right frontal lobe and displaying high signals on both T2WIs and FLAIR sequences ,isointensity of T1WIs with no evidences of restricted diffusion seen herein.
- No other cerebral, cerebellar or brain stem areas of altered MR signal/abnormal enhancement/foci of restricted diffusion.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Cuts taken in the para-sellar regions are unremarkable.
- Flattened the pituitary gland towards the flood of the dorsum sellae,measuring about 2.9mm in its height and the rest of dorsum sellae is replaced with CSF-signal.
- Cuts taken in the orbits are unremarkable.
- **Cuts taken in the para-nasal sinuses revealed mild ethmoidal and left sphenoidal sinusitis with a tiny polyp seen in the left maxillary antrum and measures about 8.2x5.9mm in dimensions.**
- **Cuts taken in the mastoid air cells revealed bilateral mastoiditis.**
- Intact cervico medullary junction.

**IMPRESSIONS:**

[[DOCTYPE:MRI]]
M.R.I

| | |
|---|---|
| **Patient's Name:** | **Maryam El-Sayed Ali Ahmed** |
| **Date:** | **2026-07-19** |
| **Referred by:** | Prof.Dr. **no doctor** |
| **Hospital:** | **Goustave Roussey International Egypt** |

- **Brain involutional changes.**
- **Two lacunar infarcts in the right frontal lobe.**
- **Flattened the pituitary gland towards the floor of the dorsum sella, and is rest of the dorsum sellae is replaced with CSF-signal,denoting a partial empty sella syndrome.**
- **Otherwise,normal study.**

Prof/Hanan El-Ahwal.